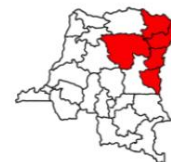




EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°096/MVEBDB/08/18/2026

📅 Reporting date: August 18, 2026

📅 Publication date: August 19, 2026



HIGHLIGHTS (last 24 hours)

At the end of the day on August 18, 2026, 103 new confirmed cases of Ebola Virus Disease (EVD), including 35 community cases, were recorded in the provinces of Ituri (82 cases), North Kivu (12 cases), and Haut-Uélé (8), and 1 in Bas-Uélé. A new health zone (Viadana) in Bas-Uélé province has been affected.

ACCUMULATIVE CASE 5,208	ACCUMULATIVE DEATH CONFIRMED 2,476	LETHALITY 47.5%	PATIENTS IN ISOLATION/CTE 730	ACCUMULATIVE HEALED 1115	TRACKING RATES CONTACTS 84.0% (Of the Day)
--	---	--------------------------------------	--	---	---



Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé



56 out of 151
health zones affected
(37.1%)



Summary of key points from the last 24 hours

In the past 24 hours, **103 new confirmed cases**, including **35 community deaths** from Ebola Virus Disease (EVD), have been reported in the provinces of **Ituri (82 cases)**, **North Kivu (12)**, **Haut Uélé (8)** and **Bas Uélé (1)**.

A new health zone has been affected in the province of Bas Uélé, namely that of Viadana, recording the second confirmed case in this province.

At the end of the day on August 18, 2026, **24 patients** were declared recovered after receiving two negative Ebola virus (EV) test results (22 in Ituri and 2 in North Kivu). On the other hand, **21 confirmed deaths within Ebola treatment centers** were recorded, including 14 in Ituri, 4 in North Kivu and 3 in Haut Uélé.

Thus, the Democratic Republic of Congo has reached a cumulative total of **5,208 confirmed cases**, including **2,476 deaths**, representing a case fatality rate of **47.5 %**. Ituri remains the epicenter of the epidemic, accounting for 84.3% of cumulative confirmed cases and nearly two-thirds (79.6%) of new cases reported in the last 24 hours.

Currently, 56 of the 151 health zones in the six provinces have been affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (2/11).

Distribution of confirmed cases and deaths by affected province

(As of August 18, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4391	1958	44.6%	28/36 (77.8%)	82
North Kivu	645	440	68.2%	12/34 (35.3%)	12
Haut-Uélé	153	69	45.1%	6/13 (46.1%)	8
Tshopo	14	7	50.0%	7/23 (30.4%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Bas Uélé	2	1	50.0%	2/11 (18.2%)	1
Total	5,208	2,476	47.5%	56/151 (37.1%)	103

Confirmed cases and deaths by province and health zone (situation as of August 18, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4391	1958	44.6%	82	29	14	43
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	8	2	25.0%				0
Aungba	11	5	45.5%				0
Bamboo	114	26	22.8%	2	0		0
Boga	1	1	100.0%				0
Bunia	1219	384	31.5%	25	12		12
Damascus	25	9	36.0%				0
Drodoro	13	6	46.2%				0
Fataki	72	30	41.7%	1	0		0
Gethy	5	2	40.0%				0
Kambala	2	0	0.0%				0
Kilo	31	12	38.7%				0
Komanda	53	37	69.8%	2	2		2
Lita	186	106	57.0%	3	2		2
Logo	11	5	45.5%				0
Lolwa	17	4	23.5%	2	0		0
Mahagi	4	1	25.0%				0
Mambasa	14	7	50.0%				0
Mandima	25	16	64.0%	2	2		2
Mangala	173	99	57.2%	8	7		7
Mongbwalu	593	282	47.6%	2	0		0
Nia-Nia	178	104	58.4%	3	1		1
Nizi	558	243	43.5%	15	3		3

Nyankunde	121	35	28.9%				0
Rimba	9	4	44.4%				0
Rwampara	879	332	37.8%	16	0		0
Tchomia	51	25	49.0%	1	0		0
To be ventilated	N / A	174	N / A			14	14
North Kivu	645	440	68.2%	12	5	4	9
Blessed	98	71	72.4%	2	0		0
Butembo	124	98	79.0%	6	2		2
Goma	1	0	0.0%				0
Kalunguta	13	6	46.2%				0
Katwa	310	204	65.8%	4	3	2	5
Kyondo	15	12	80.0%				0
Lubero	1	1	100.0%				0
Mabalako	2	1	50.0%				0
Masereka	7	4	57.1%				0
Musienene	63	37	58.7%			2	2
Oicha	6	4	66.7%				0
Vuhovi	5	2	40.0%				0
Haut-Uélé	153	69	45.1%	8	1	3	4
Boma Mangbetu	23	9	39.1%			1	1
Gombari	1	1	100.0%				0
Isiro	55	24	43.6%	8	1	1	2
Pawa	20	14	70.0%				0
Rungu	1	0	0.0%				0
Wamba	53	21	39.6%			1	1
Tshopo	14	7	50.0%	0	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	3	2	66.7%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	5	4	80.0%				0
Mangobo	2	1	50.0%				0
Tshopo	1	0	0.0%				
Wanie-Rukula	1	0	0.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	2	1	50.0%	1	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%	1	0	0	
Total	5208	2476	47.5%	103	35	21	56

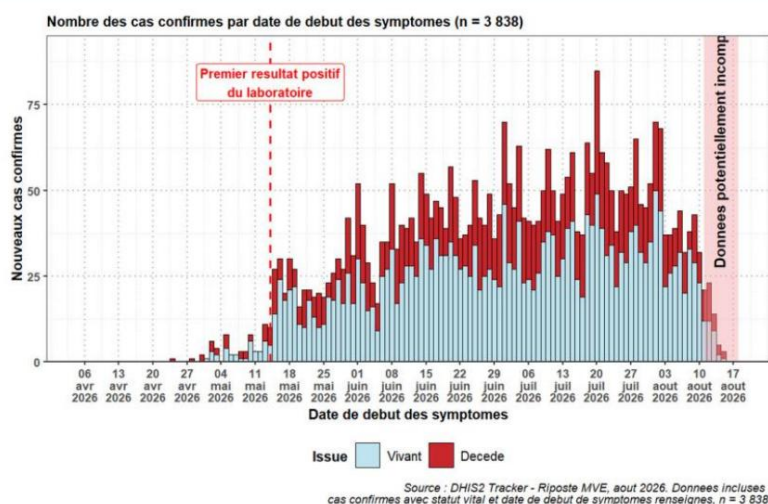
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

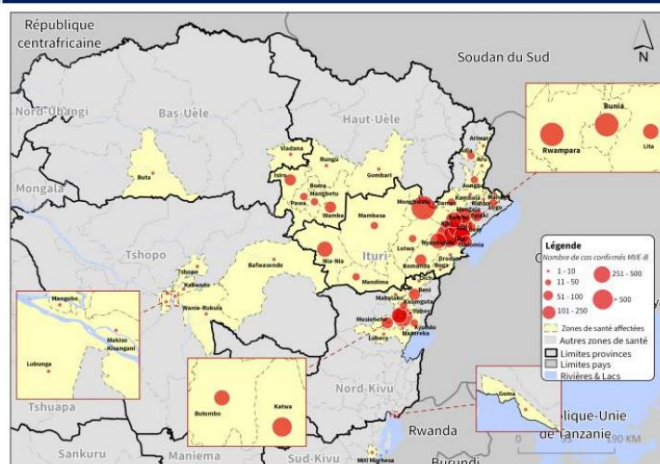
Status of alerts notified by province, as of August 18, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (as of today)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	14	7	21	14	7	0	0	21	14
Bas Uélé	4	1	5	4	1	0	0	5	1
Ituri	653	88	741	132	61	353	5	193	126
North Kivu	883	61	944	89	61	786	0	149	89
South Kivu	18	0	18	9	0	9	0	9	6
Tshopo	109	2	111	11	0	94	0	11	1
Total General	1681	159	1,840	259	130	1242	5	388	237

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : evaluation visit of care at the Kitumba CTE with the Ugandan team and meetings with the sub-coordinations of Kyondo and Katwa.
- **Ituri/Tshopo** : Joint MVE supervision at the central/DPS level in the four zones of urban health (Mangobo, Tshopo, Kabondo and Makiso-Kisangani).
- **Bas-Uélé**: meeting with the Minister of Health regarding the Ebola death alert in the health zone Likati and joint raid with the prosecutor's office in Buta made it possible to overcome the resistance to decontamination.

1.2. Epidemiological surveillance

1.2.1. Main Actions

By the end of the day on August 18, 2026, 1,840 alerts had been recorded, including 1,636 (88.9 % were verified in the six provinces. Of these alerts, 389 (23.8%) were validated as suspected cases and 388 (99.7%) were investigated, of which 237 were transferred to CT/CTE.

The proportion of contacts monitored in the last 24 hours has risen to 84.0% (18,517 seen out of 22,043 to be monitored), although it remains below the 85% threshold: Ituri 87.7 % (11,229/12,809), Tshopo 75.5% (336/445) and North Kivu 82.4% (6,206/7,525). Haut-Uélé 59.1% (746/1,262) and Bas-Uélé 18.7% (36/191).

1.2.2. Challenges

Poor contact tracing around confirmed cases mainly in the provinces of Tshopo and Bas Uélé linked to insufficient response teams and community resistance in some areas.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- Six (6) alerts were detected at PoE/PoC during the last 24 hours: four in Ituri, including two live alerts validated and referred to the CTE (PoC Mudzibala in Bunia and PoC Nizi, 2/2) and two lifeless bodies intercepted on the Watsa road (Aru SZ), of which only one could be swabé (result returned negative); one in North Kivu (PoC Foner Kasindi), invalidated after thorough investigation and one in Haut-Uélé (not validated).

Monitoring of indicators at PoE/PoC by province, as of August 18, 2026

Indicators	Ituri	North Kivu	Haut-Uélé	South Kivu	Overall
Proportion of PoC enabled and PoE enhanced	100%	100%	36.4%	100%	92.1%
Number of people who have switched to PoE/ PoC	147,918	97,087	5,780	7,660	258,445
% of travelers screened	98.0%	97.5%	99.4%	97.5%	97.8%
% of travelers who washed their hands	97.7%	97.5%	99.2%	97.5%	97.7%
% of travelers made aware	99.0%	97.5%	99.2%	100%	98.5%
Number of alerts notified	4	1	1	0	6
Number of alerts alive validated	2	0	0	0	2
Number of bodies recovered today	2	0	0	0	2
Number of problematic contacts intercepted today	0	0	0	0	0
Percentage of validated live alerts referred to the CT/CTE	100%	N / A	N / A	N / A	100%
% of lifeless bodies swabed	50%	N / A	N / A	N / A	50%

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: in Haut-Uélé, only 11 entry points are activated out of the 38 priority points (28.9%) and 5 out of 11 have reported (45.5% completeness), due to a lack of training, supervision and logistical and financial support.
- Constraints affecting the functionality of PoE/PoC: non-payment of service providers in North Kivu and strike in Haut-Uélé which deprived six PoCs (Dusu, Apodo, Aungba, PK6 Route Niangara, Gossamu and Magambe) of reporting, with a break in inputs; refusal of screening in North Kivu (2,383; 2.73%) and Haut-Uélé (58; 1%), motivated by denial of the disease, fear of contamination by water and fear of internment at the CTE; non-compliance with health requirements at PoE/PoC by some officials.

1.4. Laboratory

1.4.1. Main actions

- **Ituri : 82 new positive results** (53 living and 29 deaths) out of 307 new samples received and analyzed (**positivity: 26.7 %**).
- **North Kivu : 12 new positive results** (7 living and 5 dead) out of 101 samples analyzed (**positivity of 11.9 %**).
- **Haut-Uélé: 8 new positive results** (7 living and 1 death) out of 15 samples analyzed (**positivity of 53.3%**).
- **Bas Uélé: 1 new positive result** (1 live sample) out of 2 samples analyzed (**positivity of 50.0%**).

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- **In Ituri**, 19 rings have been opened out of 74 expected (25.7%) and 76 places of residence have been decontaminated out of 96 identified (79.2%), with 14 handwashing stations installed in Nizi ; out of 103 death alerts received, 56 bodies were collected by swab, 6 failed and 79 EDS carried out.
- **In North Kivu**, 3 rings were opened in Beni, 3 FOSA and 4 households decontaminated and 30 PCI/ WASH kits distributed in Katwa; 126 FOSA were supported and 468 providers briefed, with completion reaching only 6 out of 12 health zones. Out of 54 death alerts, 53 bodies were collected by swab (1 failure due to community resistance in Beni) and 56 EDS carried out, with deaths concentrated in Katwa (37.3%) and Beni (29.6%).
- **In Bas-Uélé**, two treatment rings have been opened (Buta and Viadana) and the decontamination of the Rubi-Lisanga health center has been completed; the decontamination of the household on Bambole Avenue is only partial, following threats from the family, which were subsequently lifted with the support of the public prosecutor's office. • **In Tshopo**, two treatment rings have been opened and the household as well as the phytotherapy center The areas frequented by the latest case have been decontaminated.
- **In Haut-Uélé**, 6 rings have been opened (5 in Isiro and 1 in Pawa), 7 healthcare facilities and households decontaminated and 6 ESS assessed; 9 swabs came back positive.

1.5.2. Challenges

- Weak implementation of IPC standards: limited ring openings in Ituri (19 out of 74) and incomplete decontamination (76 out of 96 sites), due to insufficient decontamination teams, missing addresses, and insecurity in some villages; resistance to EDS in Ituri (AS PK51, Bambu, and Mangala) and family refusal in Komanda; insufficient IPC supplies and water supply difficulties in Bas-Uélé, where the pillar

has no means of transportation; very low PCI scores in several ESSs in Tshopo (11.6% at Clinique William, 12.6% at CS Papa Freddy).

1.6. Continuity of care

1.6.1. Main actions

- **In Ituri**, 70 new admissions and 2 readmissions were recorded, with 95 discharges, including 22 recoveries. Thus, 447 patients are hospitalized out of 965 beds, representing an overall occupancy rate of 46.3%. Thirty-three out of 35 ambulances are available, with 14 in use and 58 out of 58 successful trips.
- **In North Kivu**, 64 admissions were recorded (9 confirmed, including 8 at the Katwa Ebola Treatment Center and 1 at the Oicha Treatment Center, and 55 suspected cases) with 40 discharges, including 2 recoveries at the Kitatumba Ebola Treatment Center. Thus, 205 patients are hospitalized in 206 beds, representing 99.5% occupancy.
- **In Haut-Uélé**, 53 patients are being treated (36 confirmed and 17 suspected); 52 beds from the Wamba and Isiro General Referral Hospitals have been allocated to the Ebola Treatment Centers and a pavilion from the Wamba General Referral Hospital has been fitted out, bringing the provincial capacity to 130 beds (occupancy 40.8%).
- **In Tshopo**, a new admission of a suspect to the CTE HC Kisangani brings to 5 the number of patients isolated for 12 beds (41.7% occupancy).
- **In South Kivu**, 20 suspected cases remain hospitalized.

1.6.2. Challenges

Low reception capacity and insufficient technical platform of several CTE/CT: saturation of the Bambu CTE (confirmed and suspected) as well as Logo and Kigonze (suspected) in Ituri and 73 patients (Kilo, Sota, Bambu, Lolwa, Mangala and Rimba) do not benefit from any food support; limited capacity of the Buta CTE, without water connection or dedicated ambulance in Bas-Uélé; in Tshopo, low capacity of the CTE, power cuts, absence of safe blood and insufficient biological technical platform; need for PPE at the Lwiro CTE (South Kivu).

Two patients escaped from the Bambu CTE due to lack of food.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, 612 new contacts were listed around the day's cases and 199 people were reached through group and individual interviews in seven health zones (Rwampara, Mangala, Bunia, Lita, Nizi, Komanda and Ariwara); a community dialogue brought together 300 inhabitants of the Kabarole group (Rwampara) and a motorized social marketing caravan was organized in Nyankunde.
- **In Ituri**, the SMSPS pillar covered 14 of the 27 new confirmed cases (51.9%) in 4 of the 12 health zones that reported cases, 29 new suspected cases, and 11 of the 22 recoveries reintegrated into the community (50%); 54 affected households received support, and 23 separated or orphaned children received community follow-up. In Tshopo, resistance to decontamination and contact tracing was overcome in Mangobo, 38 people received psychoeducation, and 818 people were reached through home visits in Bondeko, Mangobo, and Losoko.
- **In Haut-Uélé**, 998 people were reached through 171 home visits and 1,011 through educational talks in the health zones of Wamba and Isiro, with advocacy efforts directed at the traditional authorities of the Makodas (Pawa) chiefdom. In South Kivu, 14,799 home visits reached 86,074 people, and 900 posters, 120 banners, and 1,500

The vests were distributed in the three urban health zones of Bukavu. North Kivu did not report.

- **In North Kivu**, mass communication and commitment of 206 elements of the PNC to the urban command headquarters of Beni for the respect of barrier measures and community engagement in the fight against Ebola ; awareness of 11 people to facilitate the decontamination of 2 structures and a household during these activities, 24 contacts agreed to be listed in the AS of Bundji (ZS Beni).

1.1.1. Challenges

- Persistent community challenges: refusal of EDS for religious convenience in Bunia (Adventist AS, 1 unresolved incident out of 3), threats from a family that prevented the complete decontamination of a household in Buta and nighttime escape of three isolated alerts in Poko (Bas-Uélé), six suspected cases not isolated in Rwampara for refusal (Ituri); low coverage of SMSPS interventions around the latest confirmed cases in Bunia (4 of the 20 notified cases); persistent needs for communication support and motivation of field actors in Tshopo.

1.8. Logistics

1.8.1. Main actions

In Ituri, 16 vehicles donated by UNICEF were received for decontamination teams in six health zones and 856 kg of PECH kits (118 packages) delivered to the Lolwa health zone, with IPC kits in Rwampara and PECH kits in the Butembo and Katwa health zones; 250 liters of fuel were made available to ambulances and response vehicles.

In Tshopo, three additional rental vehicles have been received and construction work continues on the isolation units in Madula and Kabondo as well as the large capacity CTE.

In North Kivu, the construction of the triage at the Vuhovi General Referral Hospital (ULB Cooperation) and the rehabilitation of the CT at the Musienene General Referral Hospital (MSF France) are continuing; PCI kits have been prepared for the health zones of Musienene and Vuhovi and UG-PDSS medicines packaged for the 11 health zones hotspots.

In Bas-Uélé, the pillar responded to requests from other pillars for the deployment of investigation teams and the transport of samples to the RadiOne site.

1.8.2. Challenges

Persistent logistical needs to support the expansion of the response: completion of the extensions of the CTEs of Nizi, ISTM and Bambu in Ituri and needs for 4 glove boxes (ZS of Fataki, Bambu-mine, Lita and Rwampara), cryotubes and cryoboxes; deployment of IPC inputs to the 7 zones of the west and north of Bas-Uélé, where the lack of Starlink kits limits communication to uncovered health areas; in Tshopo, mobility deficit (22 motorcycles available out of 200 required), shortages and near-shortages of inputs and needs for communication units for the 56 members of the SGI.

1.9. Security

1.9.1. Main actions

- In Bas-Uélé, the security pillar contacted the prosecutor, who issued a request for information to the judicial police officer; a joint team (prosecutor's office, security pillar, and ECZS of Buta) intervened to overcome the family's resistance and begin the decontamination of the home. Challenge: North Kivu, Ituri, Haut-Uélé, and Tshopo did not submit security data for the period under review.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Continued awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA) in the affected provinces; in South Kivu, 30 humanitarian actors were trained on Ebola Virus Disease (EVD), CREC (Critical Response to Sexual Exploitation), accountability to affected populations, and PSEA in the three urban health zones of Bukavu. No consolidated daily data was transmitted by the pillar at the national level.

1.1.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse is still insufficient in all areas concerned by the response.

KEY MESSAGES	TIMELINE OF KEY EVENTS
ÿ Ebola virus transmission remains active, with 103 new cases including 35 deaths community and 21 intra-CTE recorded in the last 24 hours, bringing the national total to 5,208 confirmed cases and 2,476 deaths. ÿ Ituri remains the epicenter of the epidemic, concentrating 84.3% of cumulative confirmed cases and 79.6% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected). ÿ Surveillance shows 99.7% of suspected cases investigated while the follow-up rate (84.0%) remains below the threshold of 85.0%. ÿ Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.	ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo. ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of 64 support teams.



EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd **Incident**

Manager: Professor

Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. Kombe NGWAMA John

Editorial Team

Professor Dr. Mulangu Katulumba Félix-Médard

Dr. Katusi Biselenge Jules

Mrs. Mayele Afua Marie

Dr. Diur Mananashi Floryan

Mr. Kamuina Kebela John

Contact:

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd